Policy pathways in healthcare

At present, we face a difficult situation in healthcare. The public sector is not effective at translating expenditures into healthcare services. Privately produced healthcare is afflicted with market failure. Health insurance companies suffer from poor financial regulation (of insurance) and from dealing with a malfunctioning healthcare system.

Greater government spending on healthcare is popular. At present, there are two pathways for this increased expenditure. More money can be put into government healthcare facilities such as primary health centres (PHCs) or government hospitals. Alternatively, more money can be put into health insurance companies who intermediate between individuals and private healthcare providers. Both these pathways work poorly. As a consequence, the welfare gains from increased government expenditure on healthcare, under the present paradigm of healthcare, are likely to be low.

The key insight lies in system thinking, in re-imagining the contractual relationships to achieve incentive compatibility. Imagine a contract between a network of healthcare providers and the patient which underwrites all healthcare for the patient for life, in exchange for fixed monthly payments. This must be not just one doctor, but a network of providers that covers all aspects of healthcare. When you get sick, you would go to your healthcare network, and they would render you healthcare services, at no additional cost. Such networks are called ‘Health Maintenance Organizations’ (HMOs).

Once this style of contracting is done, the incentives of the healthcare producer change. Now, the HMO is paid by you every month, and these payments are clean profit for them, *until you get sick*. When you get sick, you impose costs upon them. They have no incentive to over-prescribe procedures, and their incentive is to keep you healthy.

Now the HMO has the incentive to ask you to come in for regular check-ups, so that problems are caught early. At all ages, immunization will be